

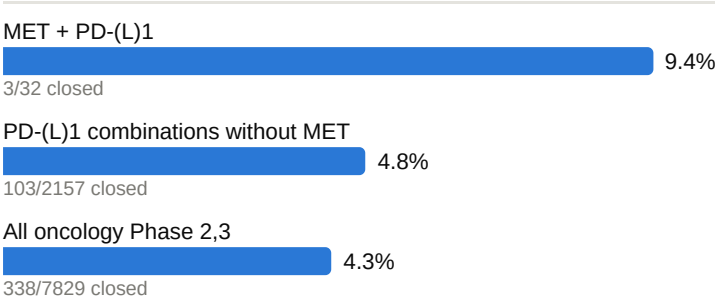
MET + PD-(L)1: 2 molecules stopped early, all small molecules

3 stopped trials, but 2 molecules — all small molecules: Capmatinib and Nintedanib. Against 4.8% for PD-(L)1 combinations without MET and 4.3% across all oncology Phase 2,3 trials in the same window. The rate counts trials that stopped early for efficacy, safety or benefit–risk reasons. Trials that ran to completion and missed their primary endpoint are counted separately below, never in the rate.

9.4% 3 of 32 closed trials (95% CI 3.2%–24.2%)	1 / 2 / 0 efficacy / safety / both (adds to 3)	30.8% of 104 trials have closed · 72 still open or unresolved	1.1% stopped for a biological reason within 3 years of starting (95% CI 0.2%–7.6%; 75 still at risk; 1.5% area-wide)
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1 of 3 **Completed and missed the primary endpoint.** Of 17 completed trials in this segment, 3 posted a primary superiority analysis on ClinicalTrials.gov; 1 came back non-significant and 1 significant, 1 split across co-primary endpoints. Counted separately and never in the rate above. The rest posted no analysis to read, so this is a floor, not a rate.

How this compares



What is in this class: 163 distinct experimental drugs across 104 trials (small molecule 103, antibody 95, unknown 9, adc 6). Most tested: CABOZANTINIB (63), NIVOLUMAB (40), ATEZOLIZUMAB (19), IPILIMUMAB (19), PEMBROLIZUMAB (15). A class groups drugs by what they act on; check that the grouping is one you would make before reading the rate as a property of the mechanism.

How much does this rest on one decision? The 3 stops came from 3 sponsor-asset programmes across 2 sponsors; the largest (Novartis) contributed 1; removing that programme's trials from both sides leaves 6.5%. Screened alongside 85 other segments in this area, it does not survive a 10% false-discovery correction (q=1, Benjamini-Yekutieli). A further 3 closed trials here were terminated with no cause recorded in the registry; if every one of them were biological the rate would be 18.8%. Read it as a screen worth checking against the underlying trials.

Bars show the share of closed trials stopped for biological reasons. Confidence intervals overlap where sample sizes are small — read the counts, not just the bars.

By start cohort

Trials started	Stops	Closed	Rate	95% CI
2015–2017	0	11	0.0%	0.0%–25.9%
2018–2020	3	14	21.4%	7.6%–47.6%
2021–2024	0	7	0.0%	0.0%–35.4%

Where the stops sit

Safety: 2, Efficacy / futility: 1.
Sponsors with most stops: Novartis (2), Jiangsu Alphamab Biopharmaceuticals Co., Ltd (1).

The stopped trials

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT04323436	2	Novartis NVS	SPARTALIZUMAB, CAPMATINIB	2022-12	Safety	Study termination based on sponsor decision due to lack of tolerability observed with capmatinib and spartalizumab combination treatment when compared to data from capmatinib single agent studies
NCT03838848	2	Jiangsu Alphamab Biopharmaceuticals Co., Ltd	ERFONRILIMAB, CARBOPLATIN, PEMETREXED, NINGETINIB	2022-07	Efficacy	Cohort A,B,C end enrollments. Cohort D and E were considered to have no significant clinical benefit at the SMC meeting and decided to terminate enrollment.

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT04139317	2	Novartis NVS	CAPMATINIB	2022-01	Safety	Sponsor decision to terminate due to the lack of tolerability observed in patients treated with capmatinib and pembrolizumab in the combination arm as compared to patients treated with pembrolizumab alone in the pembrolizumab single agent arm

Method

Denominator: ClinicalTrials.gov interventional Phase 2,3 oncology trials started 2015–2024 that have closed (completed or terminated).
Numerator: terminated trials whose registry stop reason is classified as biological (efficacy, safety or benefit–risk) by Classification V2 — held-out precision 95.5%, recall 95.3% (n=600). Drugs are linked to ChEMBL and the NCI Thesaurus; 76% of industry oncology trials in this window carry a resolved drug target, and a trial without one cannot enter a mechanism class. Intervals are Wilson 95%.

Limits

Not a failure rate: completed trials that missed their endpoints are not in the rate. They are counted separately where the sponsor posted a primary analysis that can be read — most did not, so that count is a floor — and programmes discontinued after a completed trial do not appear. Stop reasons are sponsor-reported. This is a closed-trial proportion, not a time-to-event analysis: only 30.8% of trials in this segment have closed, and a trial that stops early enters the denominator sooner than one that runs to completion, which can inflate the rate in immature segments. Recent cohorts have fewer closed trials, so their rates are less stable. Research signals, not clinical or investment advice.

The whole cohort, trial by trial. The evidence package names every trial behind these figures — the ones still running and the ones that completed and missed their primary endpoint included — and places your molecule against each one that failed. €99, delivered immediately: clinicaltrialfailures.com/asset-check

Sources: ClinicalTrials.gov (NLM); ChEMBL (EMBL-EBI, CC BY-SA 3.0); NCI Thesaurus (NCI); RxNorm/RxClass (NLM); SEC EDGAR. Classification, linkage and rates by Clinical Trial Failures. Rebuilt weekly; this brief covers trials started 2015–2024.